

CLINICAL PRACTICE

Clinical Documentation Pack: Intake, Treatment Plan & Progress Notes

Templates and worked examples for the whole clinical record, written for the era of patient portals, open notes and payer audits.

AUDIENCE	FORMAT	VERSION	TIME TO ADAPT
Psychologists · Psychiatrists	Templates + examples	1.0 · Sept 2026	1 hour

WHAT IT IS	A complete set of clinical-record templates: biopsychosocial intake with a mental status exam, a SMART treatment plan, and progress notes in SOAP, DAP and BIRP formats with short worked examples.
USE IT WHEN	You are opening a practice, changing EHR, tightening documentation for audits, or adapting to patients reading their own notes.
CUSTOMIZE FIRST	Your board's documentation rules, payer requirements, your EHR's fields, and which measures you use.

Part A: Write for the reader you will have

Assume the patient will read your notes. Patient portals, the 21st Century Cures Act's information-blocking rules, and a patient's general HIPAA right of access all push in that direction, and a payer or court may read them too. Good notes are accurate, specific, respectful and no longer than they need to be.

	Progress notes (the record)	Psychotherapy notes (HIPAA)
What they are	The clinical record of care: session times, modalities and frequencies, results of clinical tests, summary of diagnosis, functional status, treatment plan, symptoms, prognosis and progress.	Notes by a mental health professional documenting or analyzing the contents of a counseling conversation, kept separate from the rest of the record.
Required?	Yes: ethics, boards and payers expect them.	No. Optional. Many clinicians keep none.
Patient access	Patient generally has a right of access.	HIPAA gives no right of access. You may share at your discretion.
Disclosure	May be shared for treatment, payment and operations under HIPAA, subject to state law.	Most disclosures require a separate specific authorization (see PA-17).
Electronic access rules	Subject to information-blocking rules if you use certified health IT, unless an exception applies (for example, substantial risk of harm).	Excluded from information-blocking requirements when they meet HIPAA's definition.

SEPARATION MUST BE REAL

To qualify as psychotherapy notes they must be kept separate from the rest of the record with real access controls. Notes stored in the same chart, or timestamps, medication data, test results, diagnoses or treatment plans placed in them, defeat the protection. If your EHR cannot separate them, you do not have separation.

Part B: Biopsychosocial intake (clinician template)

Patient name or ID

Date of assessment

Clinician

Referral source and reason

Domain	What to capture
Presenting problem	In the patient's words. Onset, course, triggers, frequency and severity. Impact on work, relationships, sleep, self-care. Why now.
Psychiatric history	Prior diagnoses, treatment (therapy, medication, hospitalization), response and side effects, adherence. Current prescriber.
Risk	Current and past suicidal ideation, plan, intent, preparatory behavior and attempts. Self-harm. Violence or homicidal thoughts. Access to firearms and other means. Protective factors. Reasons for living.
Substance use	Alcohol, cannabis, nicotine, stimulants, opioids, prescription misuse and other. Quantity, frequency, consequences, prior treatment.
Medical	Conditions, medications, allergies, pain, sleep, head injury, pregnancy status, primary care contact. Ask about anything that could mimic or worsen psychiatric symptoms.
Family history	Psychiatric, substance and medical history in relatives, including suicide. Ask about family strengths as well.
Development and education	Early development, learning, school experience, milestones.
Trauma and adversity	Screen sensitively. Ask whether there is anything the patient wants you to know, rather than requiring detail at intake. Assess current safety.
Relationships and supports	Partner, family, friends, community. Recent losses. Isolation.
Work, finances and legal	Employment, financial stress, housing, legal issues, military service.
Culture, identity and values	Culture, language, religion or spirituality, gender identity, sexual orientation, experiences of discrimination, and how the patient wants these considered.
Strengths and goals	What has helped before, what the patient is proud of, what they want from treatment.

Mental status examination

Area	Observations
Appearance and behavior	
Speech	
Mood (patient's words) and affect	
Thought process	
Thought content (including SI, HI, delusions, obsessions)	
Perception (hallucinations)	
Cognition and orientation	
Insight and judgment	

Measures administered and scores

Diagnostic impression (with ICD-10-CM codes)

Case formulation: predisposing, precipitating, perpetuating and protective factors

Level of care and rationale

Initial plan and next appointment

Part C: Treatment plan

A useful plan has few goals, measurable objectives, named interventions, a frequency, a way to measure progress, and a review date. The patient takes part in writing it, and their words appear in it. Review at least every [90] days or when clinical status changes.

Problem / goal	Measurable objective	Interventions	Frequency	Measure	Target date
1. Reduce panic episodes	Panic episodes (patient log) fall from about 4 per week to 1 or fewer per week	Interoceptive exposure; cognitive restructuring of catastrophic beliefs; breathing retraining	Weekly, 53 min	PDSS or GAD-7 every 4 sessions	[date, about 12 weeks]

MEDICAL-NECESSITY LANGUAGE THAT PAYERS LOOK FOR

State the diagnosis, the specific symptoms and functional impairment, why this level and frequency of care is needed, what you expect to change, and how you will know. Example: "Patient meets criteria for panic disorder with frequent episodes causing missed work and avoidance of driving. Weekly psychotherapy is necessary to reduce episode frequency, restore driving and reduce functional impairment, measured by episode log and GAD-7."

Patient signature (participation)

Clinician signature

Date

Part D: Progress note templates and examples

Every note should show: date, start and stop times, modality and location (for telehealth, the patient's location and platform), who was present, what was addressed, risk, interventions used, patient response, progress toward goals, and the plan. Choose one format and use it consistently. Examples below are fictional.

SOAP

S: Patient's report and concerns in their words. **O:** Clinician's observations, MSE, measures. **A:** Assessment of progress, risk, diagnosis. **P:** Interventions planned, homework, next appointment.

Example. Start 10:02, end 10:53 (51 min), video, patient at home in [state]. **S:** Reports two panic episodes this week, one at work, rated 6 and 7 out of 10. Sleeping about five hours. **O:** Alert, cooperative, mood "anxious," affect congruent, thought process linear, no SI or HI. GAD-7: 12 (previously 15). **A:** Partial response. Panic frequency decreasing. Risk low. Continues to avoid driving. **P:** Continue weekly CBT with interoceptive exposure. Homework: daily practice and episode log. Next session [date]. 90834.

DAP

D: Data: what the patient and clinician reported and observed. **A:** Assessment. **P:** Plan.

Example. **D:** Patient described an argument with a family member and feeling "on edge" for two days. Used the paced-breathing skill once with partial relief. Reports drinking two glasses of wine on three nights. **A:** Stress is exacerbating anxiety and sleep. Alcohol may be worsening symptoms. Risk low. **P:** Review alcohol use in next session. Continue skills practice. Consider sleep hygiene plan. Next appointment [date].

BIRP

B: Behavior: presenting behavior and concerns. **I:** Intervention: what you did. **R:** Response: how the patient responded. **P:** Plan.

Example. **B:** Patient arrived tearful after a job loss and described low motivation and poor sleep. Denied SI. **I:** Behavioral activation: scheduled three small activities and reviewed barriers. Reviewed safety plan. **R:** Patient became engaged, identified two activities, and rated hope 5 out of 10, up from 3. **P:** Weekly sessions continue, review activity log next session, PHQ-9 in two sessions.

Part E: Note-quality checklist

☐ Date, start and stop times, modality and location documented.

- ☐ Risk documented, including when there is none: “denied SI, HI”.
- ☐ Interventions named specifically (“behavioral activation”, not “supportive”).
- ☐ Response to treatment and progress toward the goal noted.
- ☐ CPT code supported by documented time and service.
- ☐ Language would still read fairly if the patient read it aloud.
- ☐ No unnecessary details about third parties, no speculation stated as fact, no judgmental labels.
- ☐ Corrections made as dated addenda, never by overwriting. Late entries labeled as such.
- ☐ Copy-forward text reviewed. Cloned notes draw payer scrutiny.

Part F: Measures with free or public terms

Check each publisher’s current terms before use. Do not reproduce a licensed instrument in your own forms.

Measure	Use	Notes
PHQ-9	Depression severity and monitoring	Free to use. Also asks about suicidal thoughts on one item.
GAD-7	Anxiety severity	Free to use.
PCL-5	PTSD symptoms	Available from the VA National Center for PTSD.
AUDIT	Alcohol use screening	Published by the World Health Organization.
ASQ (Ask Suicide-Screening Questions)	Brief suicide-risk screen	Provided by the National Institute of Mental Health.
C-SSRS	Suicide severity rating	Free for clinical use from Columbia; obtain from the source with training.

IN PSYALLIANCE

The shared World library is the right place to swap note templates, treatment plan examples and measure sets that work in your specialism. Never upload documents with real patient information.

Sources and further reading

- HHS, HIPAA Privacy Rule: psychotherapy notes, 45 CFR 164.501 and 164.508(a)(2) ([hhs.gov/hipaa](https://www.hhs.gov/hipaa)).
- HHS ONC, Information blocking and the 21st Century Cures Act ([healthit.gov](https://www.healthit.gov)).
- APA Record Keeping Guidelines ([apa.org/practice/guidelines/record-keeping](https://www.apa.org/practice/guidelines/record-keeping)).
- AMA CPT time rules for psychotherapy codes 90832, 90834 and 90837.

ABOUT THIS DOCUMENT

Prepared by PsyAlliance from publicly available professional and regulatory sources, reviewed against them as of September 2026. It is an educational template and practice aid. It is not legal, ethical, clinical or billing advice, and it does not create a professional relationship. Laws, licensing-board rules and payer requirements vary by state and change over time. Verify against your licensing board, professional association, payer contracts and, where the stakes warrant it, a qualified attorney before relying on it.

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